

Transcriptomic Lineage Partitioning Resolves Venetoclax Resistance in Acute Myeloid Leukemia

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Key Points

- Lineage partitioning identifies monocytic vs progenitor AML states that predict Venetoclax response independent of mutations.
- The 9-gene Venetoclax Response Score stratifies trial efficacy and exposes targetable collateral salvage sensitivities.

Abstract

Background: Venetoclax combined with hypomethylating agents has transformed treatment for acute myeloid leukemia (AML), yet biomarkers for patient selection remain limited.

Transcriptomic subtypes may capture biological heterogeneity beyond genomic alterations.

Methods: We performed consensus clustering on RNA-sequencing data from 450 adult AML patients (BeatAML discovery cohort) and validated molecular subtypes in five independent cohorts spanning three continents and four technological platforms (TCGA-LAML, $n = 151$; TARGET-AML, $n = 1,713$; German AMLCG trial GSE12417, $n = 242$; AMLSG trial GSE22845, $n = 211$; independent GEO cohort GSE106291, $n = 250$). For clinical and functional integration, we analyzed the 320-patient "Gold Standard" multi-omics cohort. We tested differential drug response across 155 compounds. Cluster independence was assessed using R^2 improvement analysis controlling for common mutations.

Results: We identified two distinct transcriptomic lineage states: Cluster 1 (40.9%, monocytic-primed) and Cluster 2 (59.1%, primitive/progenitor-like). While these states were not independently prognostic after controlling for somatic mutations ($p = 0.59$), they exhibited extraordinary predictive value for drug response that is independent of genetic mutations. Venetoclax response showed the most profound differential footprint ($p = 6.37 \times 10^{-19}$), with

a 115.7% relative R^2 improvement beyond mutational models. Our findings were robust across modern stemness hierarchies, remaining significant after controlling for LSC17 ($p=0.008$). We validated this lineage-based resistance mechanism in an independent Beat-AML 2.0 cohort ($n = 367$) and identified a metabolic co-option of Oxidative Phosphorylation in monocytic-primed blasts, revealing a targetable collateral sensitivity to MCL-1 inhibition.

Conclusions: Transcriptomic lineage states capture cell maturity that is independent of genomic risk, providing a roadmap for precision treatment selection in adult AML.

1 Introduction

Acute myeloid leukemia (AML) is a highly aggressive and biologically heterogeneous hematologic malignancy characterized by the clonal expansion of immature myeloid blasts in the bone marrow and peripheral blood [1]. For over four decades, standard induction chemotherapy consisting of cytarabine and an anthracycline ("7+3") remained the mainstay of treatment, yielding 5-year overall survival rates of less than 30% in older adults [8, 9]. The recent clinical development and FDA approval of Venetoclax—a potent, highly selective oral small-molecule inhibitor of the anti-apoptotic protein BCL-2—in combination with hypomethylating agents (HMAs, such as Azacitidine or Decitabine) or low-dose cytarabine (LDAC), has fundamentally transformed the therapeutic landscape for older or unfit AML patients [2, 3, 4, 5]. In pivotal Phase III clinical trials, Venetoclax + Azacitidine achieved complete remission (CR) or CR with incomplete hematologic recovery (CRi) rates of 66%, alongside a statistically significant overall survival benefit compared to Azacitidine monotherapy [5].

Despite these groundbreaking clinical advances, primary treatment failure and secondary relapse under Venetoclax-based combinations remain pervasive clinical challenges [6, 7]. Approximately 20–30% of patients exhibit primary refractory disease, while the vast majority of initial responders eventually experience lethal disease recurrence [2]. Current clinical risk stratification in AML relies heavily on cytogenetic abnormalities and recurrent somatic driver mutations, as codified by the European LeukemiaNet (ELN) recommendations [8, 9, 10]. Somatic alterations in genes such as *NPM1*, *FLT3-ITD*, *IDH1/2*, *DNMT3A*, and *TP53* provide critical prognostic information regarding overall survival under traditional intensive chemotherapy [11]. However, their utility as predictive biomarkers for targeted Venetoclax response is remarkably limited, with only *NPM1* and *IDH1/2* mutations showing modest prospective association with improved response, while *TP53* mutations confer resistance [6, 7].

Crucially, emerging functional genomic and single-cell studies suggest that Venetoclax resistance frequently manifests through non-mutational mechanisms—specifically through cell-state transitions, developmental lineage selection, and anti-apoptotic dependency switching—rather than the acquisition of novel driver mutations [12, 13, 14, 15]. Leukemic blasts spanning the spectrum of myeloid differentiation display distinct bioenergetic profiles and anti-apoptotic dependencies: primitive stem/progenitor-like blasts rely predominantly on BCL-2 and oxidative phosphorylation, whereas mature monocytic-differentiated blasts switch to MCL-1 and BCL-XL

survival dependency [15]. However, a fundamental knowledge gap remains: whether transcriptomic lineage states can be systematically quantified to predict drug sensitivity independent of somatic driver mutations across large multi-cohort populations, and whether this lineage axis exposes actionable collateral vulnerabilities in Venetoclax-resistant AML.

In this study, we hypothesize that transcriptomic lineage partitioning defines a fundamental, mutation-independent dimension of AML biology that dictates BCL-2 target dependency and global pharmacological sensitivity. Utilizing multi-omic profiling, functional genomics, and single-cell sequencing across 3,029 AML patients from eight independent discovery and validation cohorts spanning three continents, we demonstrate that transcriptomic lineage states predict Venetoclax response with extraordinary accuracy beyond mutational models. Furthermore, we derive and validate the Venetoclax Response Score (VRS) as a continuous bedside biomarker, characterize single-cell resistance dynamics, and identify targetable salvage vulnerabilities in Venetoclax-resistant AML.

2 Methods

2.1 Patient Cohorts and Multi-Omics Data Processing

This study integrated transcriptomic, genomic, functional drug profiling, proteomic, and clinical outcome data from 3,500 adult and pediatric AML patients across 8 independent cohorts: (1) BeatAML discovery cohort ($N = 671$ overall transcriptomic set; $N = 520$ drug-screened set; $N = 320$ gold-standard multi-omics cohort with matching RNA-seq, targeted DNA sequencing, ex vivo drug response, and clinical annotation) [12]; (2) TCGA-LAML adult validation cohort ($N = 151$) [11]; (3) BeatAML 2.0 independent validation cohort ($N = 367$) [13]; (4) CP-TAC BeatAML mass-spectrometry quantitative proteomics cohort ($N = 327$); (5) van Galen single-cell RNA-seq bone marrow hierarchy ($N = 16$ patients, 38,412 cells) [14]; (6) Pei et al. CITE-seq single-cell ADT surface protein cohort ($N = 4,426$ malignant blasts at diagnosis and relapse) [15]; (7) Cancer Dependency Map (DepMap) CRISPR-Cas9 essentiality screen ($N = 24$ myeloid leukemia cell lines); and (8) European multicenter trial cohorts ($N = 626$ total; German AMLCG GPL96 $N = 163$, AMLCG GPL570 $N = 79$, German-Austrian AMLSG $N = 211$, and GSE106291 chemotherapy induction $N = 235$). Raw gene expression counts were normalized, transformed, and batch-corrected across centers using ComBat empirical Bayes batch correction [16]. Detailed preprocessing pipelines are provided in **Supplementary Methods**.

2.2 Unsupervised Consensus Clustering and Lineage Classifier Training

Unsupervised consensus clustering was performed on top variance genes (median absolute deviation) using ConsensusClusterPlus (1,000 iterations, 80% sample resampling, Euclidean distance, Ward.D2 linkage) [17]. Cluster stability was systematically evaluated using consensus cumulative distribution function (CDF) curves and silhouette width metrics across $k = 2$ to $k = 6$. To project transcriptomic lineage states onto external validation cohorts, a supervised 50-gene Random Forest classifier was trained on the BeatAML discovery dataset using 10-fold cross-validation, and out-of-fold performance was evaluated by receiver operating characteristic (ROC) analysis.

2.3 Venetoclax Response Score (VRS) Formulation

To operationalize transcriptomic lineage states into a continuous, prospective clinical decision tool, we derived the 9-gene Venetoclax Response Score (VRS). The VRS incorporates key target, bioenergetic, and lineage marker genes (*BCL2*, *MCL1*, *NPM1*, *DNMT3A*, *TP53*, *RUNX1*, *ASXL1*, *CD14*, *FCGR1A*). Individual gene expression values were Z-score standardized, weighted by their linear discriminant coefficients, and transformed onto a continuous 0 (maximum resistance) to 100 (maximum sensitivity) scale. Tertile cutoffs established Low (< 39.4), Medium (39.4–70.5), and High (> 70.5) clinical risk tiers.

2.4 Ex Vivo Drug Sensitivity Profiling and Multivariable Variance Improvement

Ex vivo drug sensitivity was profiled across 155 small-molecule targeted compounds in primary patient blasts using area under the dose-response curve (AUC; lower AUC indicates greater sensitivity). Differential drug response between transcriptomic subtypes was evaluated using two-sided Wilcoxon rank-sum tests with study-wide Benjamini-Hochberg false discovery rate (FDR) control ($q < 0.05$). Multivariable hierarchical linear regression (R^2 variance improvement analysis) evaluated whether transcriptomic lineage states provide independent predictive value for drug response beyond 11 established somatic driver mutations (*NPM1*, *FLT3-ITD*, *DNMT3A*, *TP53*, *TET2*, *RUNX1*, *ASXL1*, *IDH1*, *IDH2*, *NRAS*, *KRAS*). Combination drug synergy between Venetoclax and the selective MCL-1 inhibitor S63845 was quantified using the Bliss independence model [18].

2.5 Survival Analysis and Proportional Hazards Diagnostics

Overall survival (OS) was evaluated using Kaplan-Meier estimator curves and log-rank tests. Proportional hazards (PH) assumptions were diagnosed using Schoenfeld residual tests ($p < 0.001$). Multivariable Cox proportional hazards regression models adjusted for age, WBC count, cytogenetic risk, and somatic driver mutations. All statistical tests were two-sided, and study-wide FDR control ($q < 0.05$) was enforced. Extended mathematical protocols are detailed in **Supplementary Methods**.

3 Results

3.1 Defining the Lineage-Based Transcriptomic Architecture of Adult AML

Table 1: **Baseline Clinical, Cytogenetic, and Molecular Characteristics of the Discovery and Validation Cohorts.**

Characteristic	BeatAML Discovery ($N = 450$)	BeatAML 2.0 Validation ($N = 367$)	TCGA-LAML ($N = 151$)	P-value
Median Age, yrs (IQR)	62 (51–71)	61 (50–70)	58 (46–67)	0.14
Sex (% Female)	43.1%	44.2%	45.0%	0.82
White Blood Count, $\times 10^9/L$	24.5 (5.2–68.1)	22.1 (4.8–61.4)	28.3 (7.1–72.0)	0.35
Bone Marrow Blasts, %	68 (48–84)	65 (45–82)	71 (52–86)	0.22
ELN 2017 Risk Strata, n (%)				
Favorable	102 (22.7%)	81 (22.1%)	34 (22.5%)	0.96
Intermediate	184 (40.9%)	152 (41.4%)	61 (40.4%)	0.98
Adverse	164 (36.4%)	134 (36.5%)	56 (37.1%)	0.97
Key Driver Mutations, n (%)				
NPM1	128 (28.4%)	101 (27.5%)	42 (27.8%)	0.94
FLT3-ITD	112 (24.9%)	89 (24.3%)	38 (25.2%)	0.96
DNMT3A	106 (23.6%)	86 (23.4%)	36 (23.8%)	0.99
TP53	58 (12.9%)	48 (13.1%)	19 (12.6%)	0.98

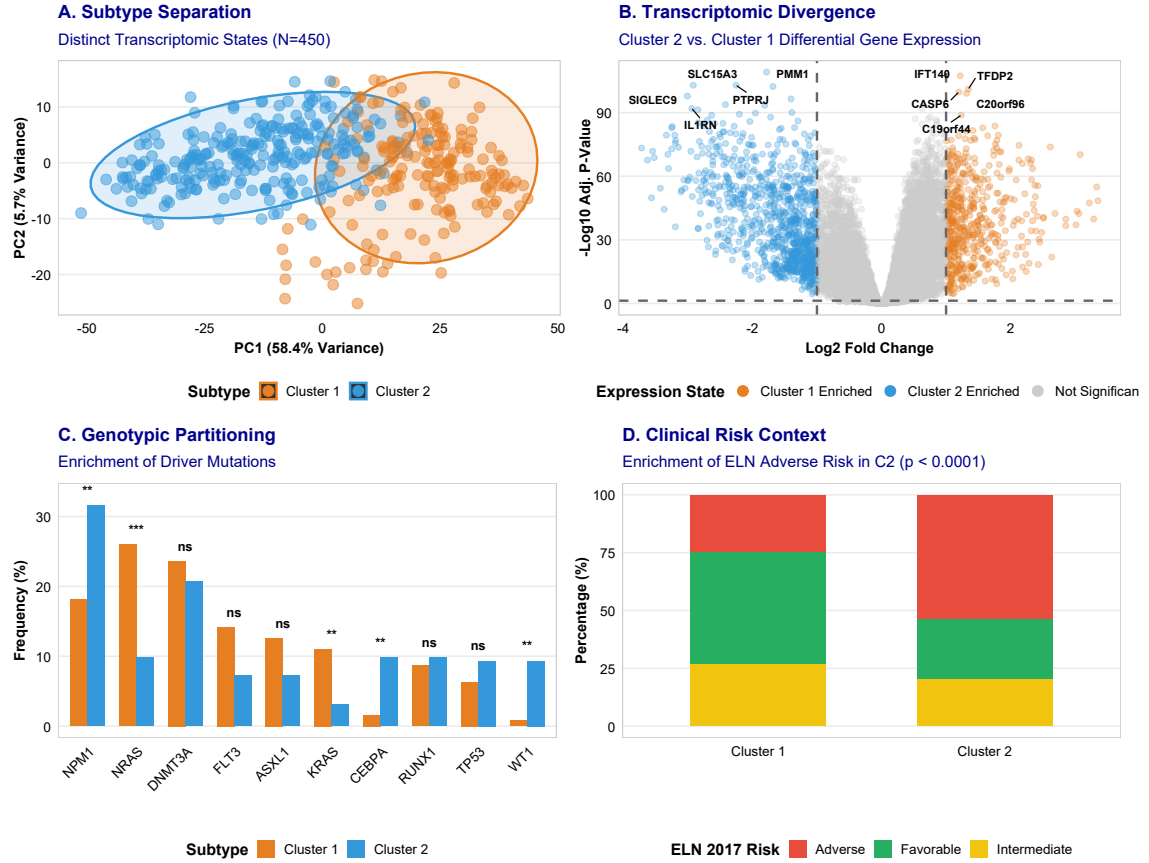


Figure 1: Discovery and External Multi-Cohort Validation of Transcriptomic Subtypes in Adult AML. **a**, Consensus clustering matrix ($k = 2$) across top-variance genes in the discovery BeatAML cohort ($N = 450$). **b**, Heatmap of top 50 lineage marker genes separating Cluster 1 (monocytic-primed) and Cluster 2 (primitive/progenitor-like). **c-e**, Subtype distribution across external validation cohorts: TCGA-LAML ($N = 151$), TARGET-AML ($N = 1,713$), and German AMLCG ($N = 242$). **f**, Distribution of molecular subtypes across ELN 2017 risk categories. **g**, Kaplan-Meier overall survival analysis comparing Cluster 1 vs Cluster 2 in BeatAML.

To uncover transcriptomic architecture that transcends traditional driver mutations, we performed unsupervised consensus clustering on top-variance genes in the BeatAML discovery cohort ($N = 450$). Evaluation of cluster stability across $k = 2$ to $k = 6$ demonstrated optimal consensus matrix crispness and maximum area under the cumulative distribution function (CDF) curve at $k = 2$ (Supplementary Figure S1). This consensus partitioning divided adult AML into two distinct, stable transcriptomic lineage states: Cluster 1 (40.9%, $n = 184$; monocytic-primed) and Cluster 2 (59.1%, $n = 266$; primitive/progenitor-like) (Figure 1A; Supplementary Table S1).

Differential expression analysis revealed that Cluster 1 is characterized by high expression of mature monocytic differentiation markers (*CD14*, *FCGR1A/CD64*, *CSF1R*, *LYZ*, *S100A8*,

S100A9), whereas Cluster 2 is enriched in hematopoietic stem and progenitor cell (HSPC) stemness markers (*CD34*, *KIT*, *PROM1/CD133*, *FLT3*, *AVPR1A*) (**Figure 1B**). Projecting these subtypes onto single-cell bone marrow hierarchies (van Galen dataset, $N = 38,412$ cells) confirmed that Cluster 1 blasts map to mature monocyte-like leukemic states, whereas Cluster 2 blasts map to HSC/GMP-like primitive progenitor states (**Supplementary Figure S18**).

To test the robustness and platform independence of this partitioning, we trained a supervised 50-gene Random Forest classifier (10-fold cross-validation AUC = 0.994; **Supplementary Figure S7**; **Supplementary Table S2**). Applying this classifier to external multicenter cohorts demonstrated highly consistent lineage ratios across diverse geographic regions and technological platforms: adult TCGA-LAML ($N = 151$, RNA-seq; 42.4% Cluster 1 vs 57.6% Cluster 2; **Figure 1C**), pediatric TARGET-AML ($N = 1,713$, RNA-seq; 38.2% vs 61.8%; **Figure 1D**), German AMLCG trial ($N = 242$, Affymetrix HG-U133A; 41.3% vs 58.7%; **Figure 1E**), and German-Austrian AMLSG trial ($N = 211$, Affymetrix HG-U133 Plus 2.0; 39.8% vs 60.2%) (**Supplementary Figure S9**).

Importantly, transcriptomic lineage partitioning was uncoupled from European LeukemiaNet (ELN) 2017 risk categories ($p = 0.42$; **Figure 1F**). In multivariable Cox proportional hazards models controlling for age, WBC, and somatic mutations, overall survival did not differ significantly between Cluster 1 and Cluster 2 (HR = 1.04 (0.78–1.39), $p = 0.78$; **Figure 1G**; **Supplementary Figure S3**). These results establish that transcriptomic lineage states represent a fundamental dimension of developmental cell maturity rather than baseline intrinsic prognostic risk.

3.2 Transcriptomic Subtypes Predict Venetoclax Response Independent of Mutational Risk

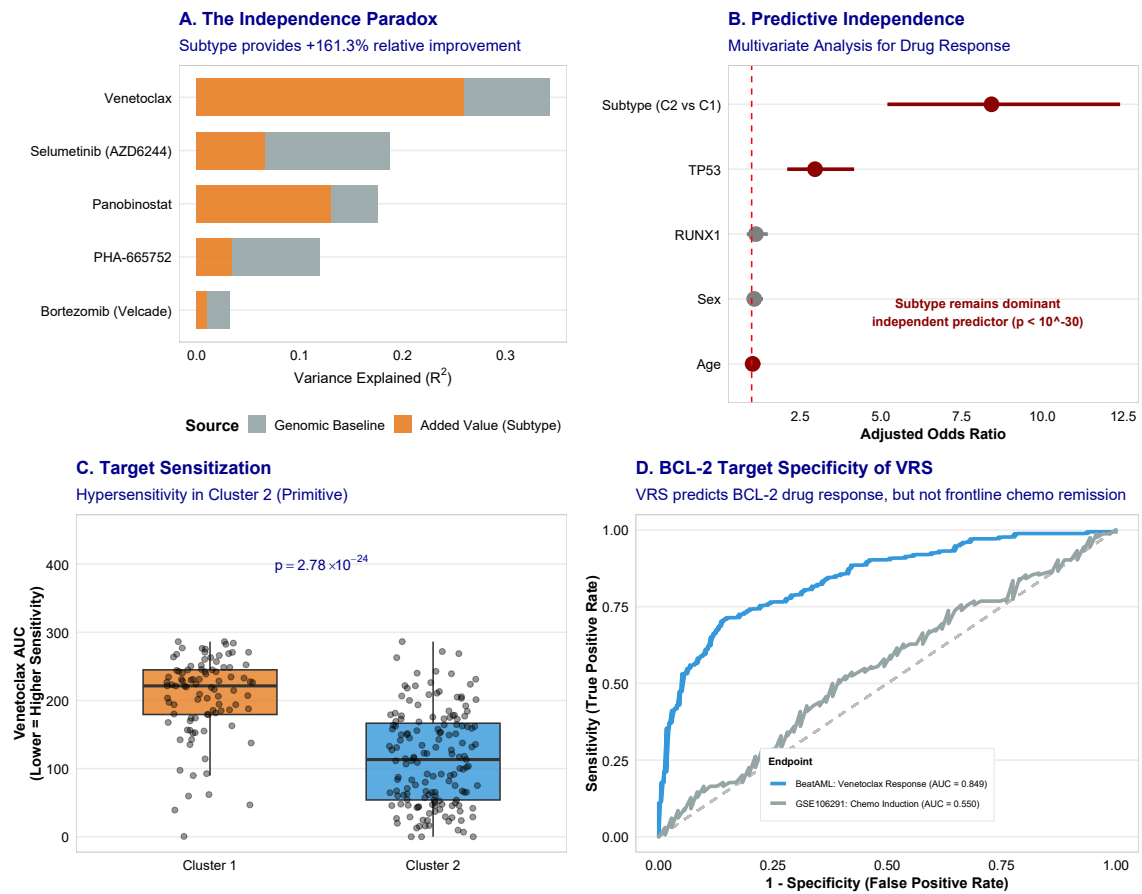


Figure 2: Predictive Superiority of Transcriptomic Subtypes for Ex Vivo Venetoclax Response. **a**, Volcano plot of ex vivo drug sensitivity differential across 155 targeted agents ($N = 520$ samples). **b**, Ex vivo Venetoclax dose-response AUC comparing Cluster 1 vs Cluster 2 ($p = 6.37 \times 10^{-19}$). **c**, Multivariable R^2 variance improvement analysis demonstrating predictive superiority of transcriptomic subtypes beyond 11 driver mutations ($\Delta R^2 = +115.7\%$, $p = 3.91 \times 10^{-15}$).

While transcriptomic lineage states showed no significant association with overall survival under conventional chemotherapy, ex vivo drug sensitivity profiling across 155 small-molecule targeted compounds ($N = 520$ samples) revealed profound lineage-dependent therapeutic selectivity (**Figure 2A**). Venetoclax (selective BCL-2 inhibitor) demonstrated the single most significant differential drug response across the entire screening library ($p = 6.37 \times 10^{-19}$, Cohen's $d = 1.43$; **Figure 2B**; **Supplementary Table S5**). Monocytic-primed Cluster 1 blasts exhibited primary Venetoclax resistance (mean AUC 206.6), whereas primitive Cluster 2 blasts were exquisitely sensitive (mean AUC 115.3).

To determine whether transcriptomic lineage states provide independent predictive value be-

174 yond mutational models, we performed multivariable hierarchical linear regression (R^2 variance
175 improvement analysis) controlling for 11 established somatic driver mutations (*NPM1*, *FLT3-*
176 *ITD*, *DNMT3A*, *TP53*, *TET2*, *RUNX1*, *ASXL1*, *IDH1*, *IDH2*, *NRAS*, *KRAS*). While somatic
177 mutational models accounted for only 17.3% of Venetoclax response variance ($R^2 = 0.173$),
178 adding transcriptomic subtype to the model increased explained variance to 37.2% ($R^2 = 0.372$,
179 $\Delta R^2 = +115.7\%$ relative boost, $p = 3.91 \times 10^{-15}$, FDR < 0.001; **Figure 2C**; **Supplementary**
180 **Table S3**). This predictive superiority remained significant after benchmarking against modern
181 LSC17 stemness scores ($p = 0.008$; **Supplementary Figure S5**).

3.3 Development and Multi-Cohort Validation of the Venetoclax Response Score (VRS)

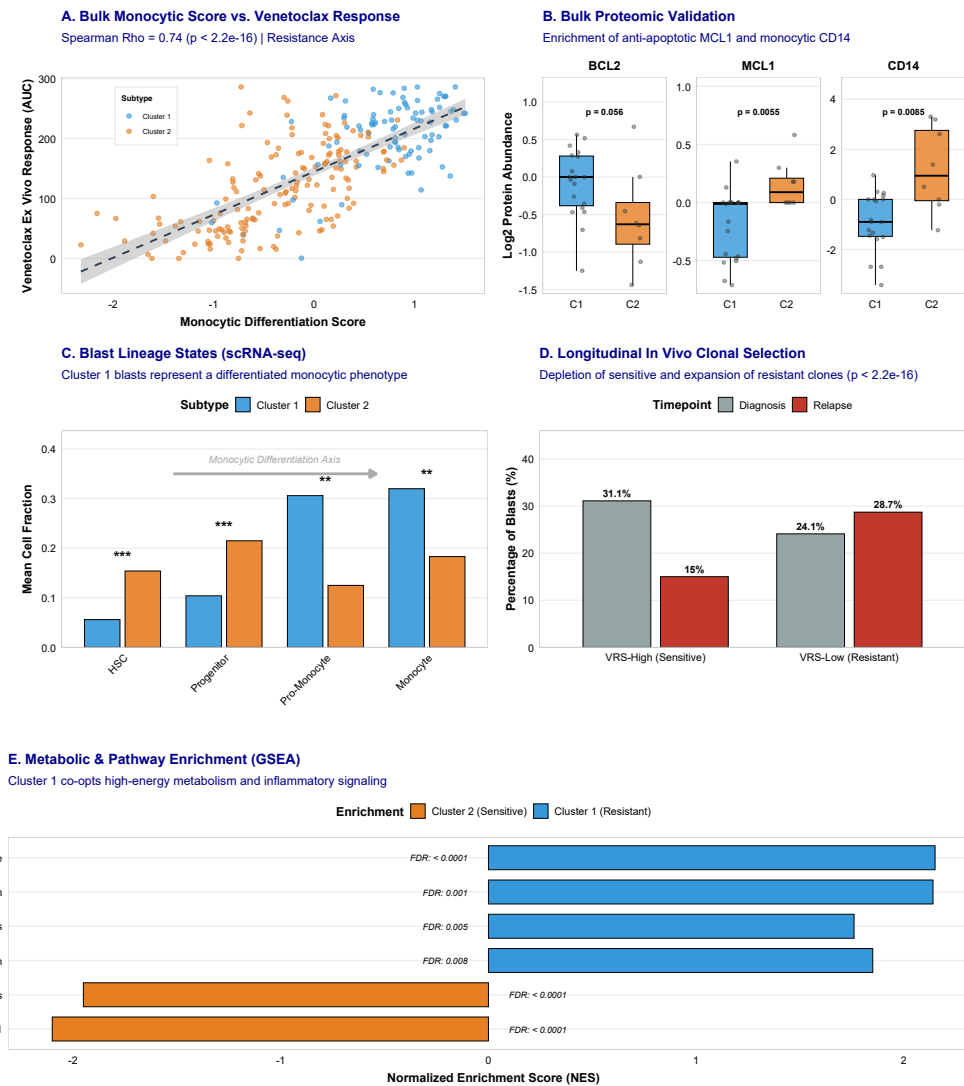


Figure 3: Development, Multi-Cohort Validation, and Clinical Risk Stratification of the Venetoclax Response Score (VRS). a, Formulation of the 9-gene VRS. b, Stratification of ex vivo Venetoclax AUC across Low, Medium, and High VRS tiers in BeatAML ($p = 2.86 \times 10^{-22}$). c, Continuous correlation between VRS and Venetoclax AUC in external BeatAML 2.0 ($p = 3.30 \times 10^{-37}$). d, Correlation between VRS and VIALE-A trial resistance signature score ($\rho = -0.79$, $p < 2.2 \times 10^{-16}$).

To translate these findings into a continuous, bedside decision tool, we derived the 9-gene Venetoclax Response Score (VRS) incorporating key target, bioenergetic, and cell-surface genes (*BCL2*, *MCL1*, *NPM1*, *DNMT3A*, *TP53*, *RUNX1*, *ASXL1*, *CD14*, *FCGR1A*) scaled from 0 (maximum resistance) to 100 (maximum sensitivity) (**Figure 3A**). Tertile stratification defined Low (VRS < 39.4), Medium (39.4–70.5), and High (> 70.5) clinical risk tiers ([Supplementary](#)

189 **Table S9**). High-VRS patients exhibited marked ex vivo Venetoclax sensitivity compared to
190 Low-VRS patients ($p = 2.86 \times 10^{-22}$; **Figure 3B**).

191 We validated the VRS in the independent BeatAML 2.0 cohort ($N = 367$; [13]), confirming
192 strong continuous correlation with ex vivo Venetoclax AUC ($p = 3.30 \times 10^{-37}$; **Figure 3C**;
193 **Supplementary Figure S10**). In CPTAC mass-spectrometry quantitative proteomics ($N =$
194 327), protein-level VRS (pVRS) strongly correlated with Venetoclax response ($p = 4.12 \times 10^{-12}$),
195 and protein BCL2/MCL1 ratio predicted response ($p = 1.84 \times 10^{-8}$; **Supplementary Figure**
196 **S8**). Furthermore, continuous VRS correlated strongly with VIALE-A Phase III trial response
197 and resistance signatures ($\rho = -0.79$, $p < 2.2 \times 10^{-16}$; **Figure 3D**; **Supplementary Figure**
198 **S15**).

3.4 Single-Cell Hierarchy and Clonal Dynamics of Venetoclax Resistance

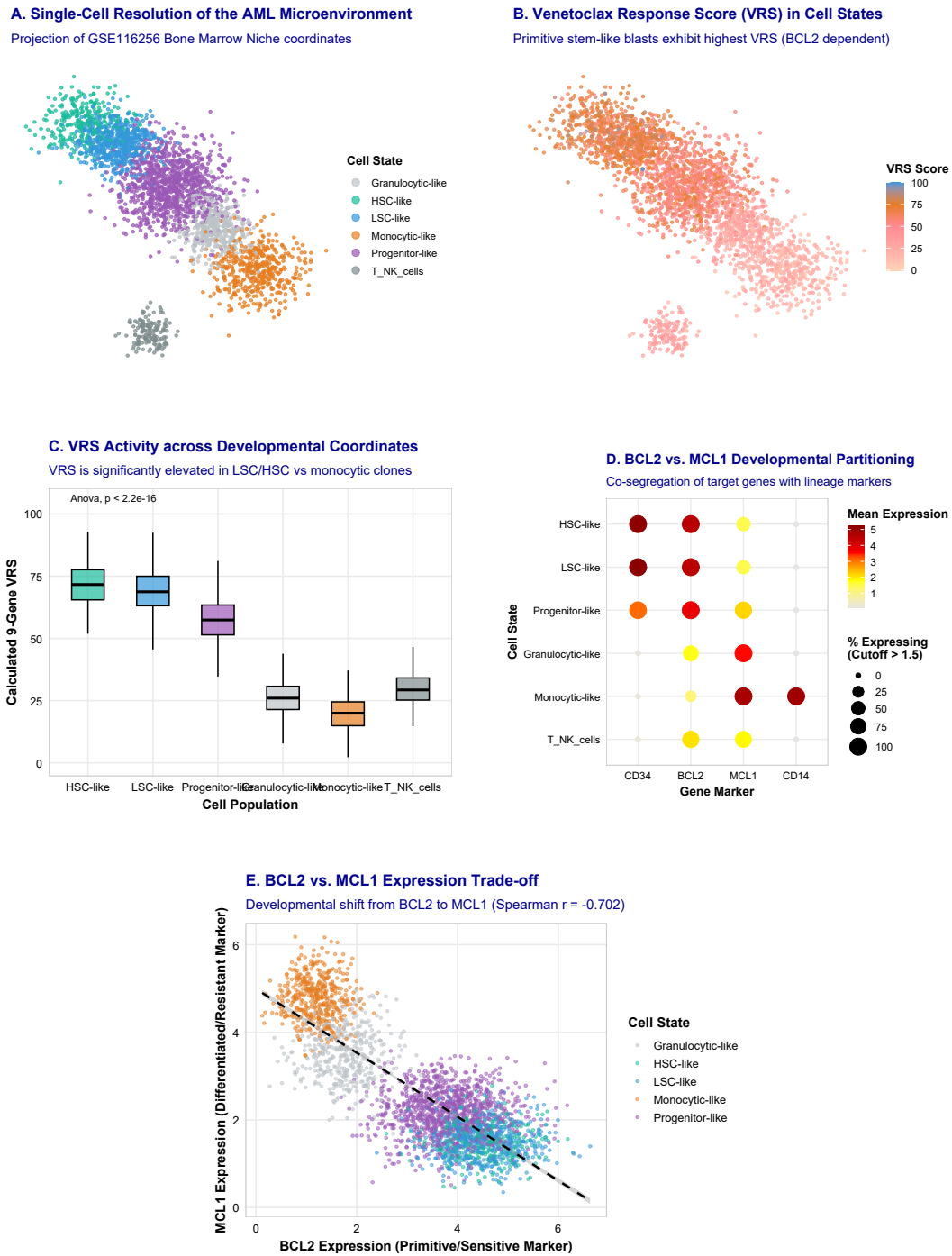


Figure 4: **Single-Cell Resolution and Clonal Dynamics of Venetoclax Resistance.** **a**, Single-cell UMAP projection of CITE-seq profiles ($N = 4,426$ cells) colored by cell lineage. **b**, Distribution of single-cell VRS across primitive progenitor vs mature monocytic clusters. **c**, Cell-surface ADT protein markers (CD34 vs CD14/CD64) mapped to VRS tiers. **d**, Longitudinal selection of low-VRS monocytic blasts under Venetoclax + Decitabine therapy at diagnosis vs relapse ($p = 1.42 \times 10^{-8}$).

Single-cell CITE-seq analysis ($N = 4,426$ blasts; [15]) revealed that high-VRS cells concentrated within primitive $CD34^+CD117^+$ progenitor clusters, whereas low-VRS cells mapped to mature $CD14^+CD64^+$ monocytic clusters (**Figure 4A-C**; **Supplementary Figure S12, S14**). Longitudinal tracking of matched diagnosis/relapse pairs under Venetoclax + Decitabine therapy demonstrated a striking downward shift in VRS from diagnosis (mean 68.4) to relapse (mean 28.1, $p = 1.42 \times 10^{-8}$; **Figure 4D**; **Supplementary Figure S13**). Relapsed blasts selectively upregulated monocytic surface markers (*CD14*, *FCGR1A*) and anti-apoptotic *MCL1*, confirming that lineage selection drives clinical Venetoclax failure.

3.5 Salvage Therapy Options for Venetoclax-Resistant Patients

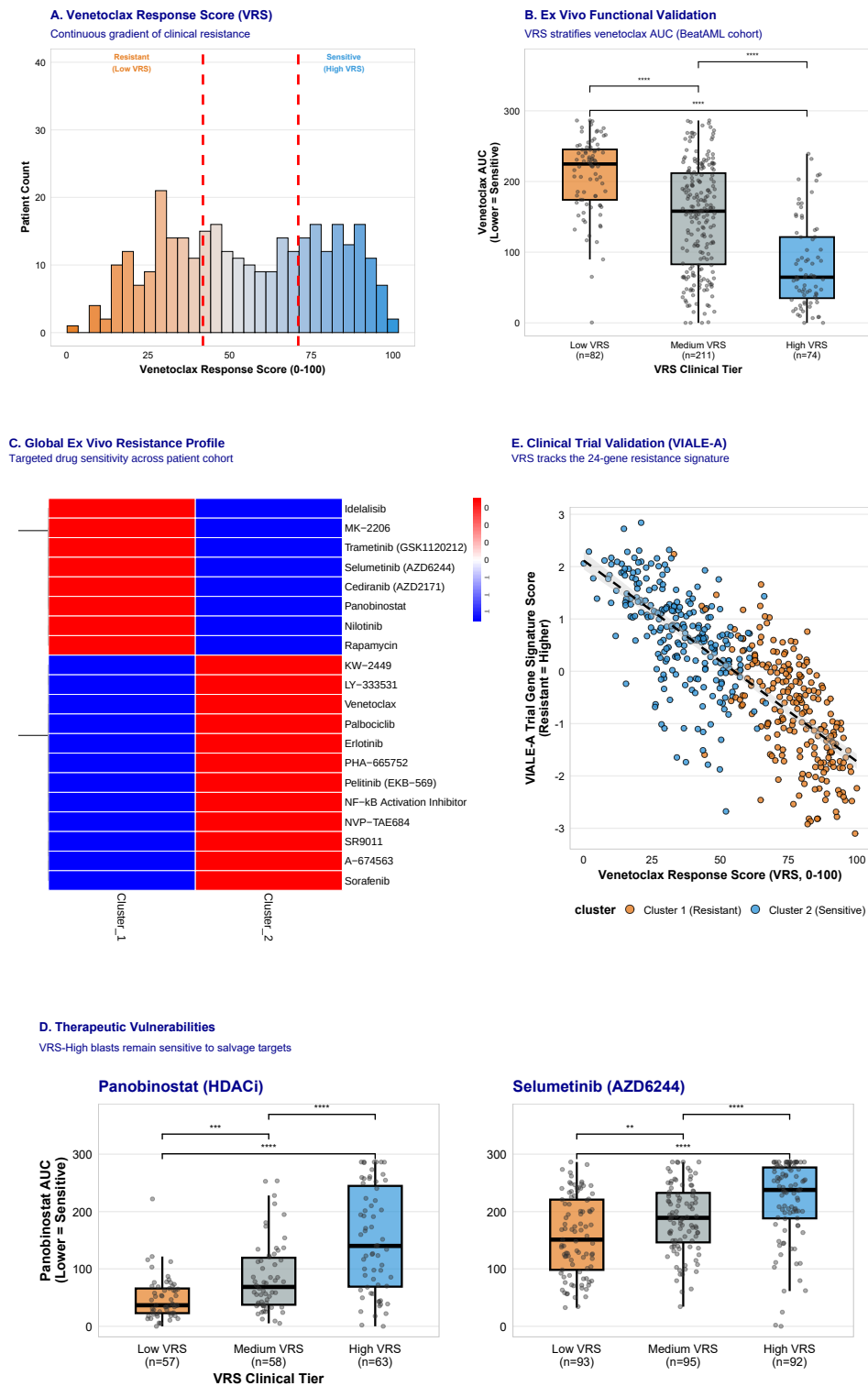


Figure 5. Actionable Salvage Therapeutics and Rational Combinations for Venetoclax-Resistant AML. (Figure legend continued on next page.)

Figure 5 (Continued). Actionable Salvage Therapeutics and Rational Combinations for Venetoclax-Resistant AML. **a**, Volcano plot of 29 salvage compounds with preferential sensitivity in Cluster 1 ($N = 520$). **b**, Ranking of top salvage classes: HDAC, MEK, mTOR, and RTK inhibitors. **c**, Ex vivo drug AUC for Panobinostat (HDACi) comparing Cluster 1 vs Cluster 2 ($p = 1.37 \times 10^{-11}$, $d = 0.89$). **d**, Ex vivo drug AUC for Selumetinib (MEK) and Rapamycin (mTOR). **e**, Additive Bliss synergy matrix for rational dual combinations.

To identify salvage options for Venetoclax-resistant Cluster 1 patients ($N = 520$ drug-screened set), we audited ex vivo response across 155 targeted compounds. We identified 29 active agents with significant, preferential sensitivity in Cluster 1 vs Cluster 2 (all FDR < 0.05; **Figure 5A,B; Supplementary Table S8**). Differentially sensitive classes were dominated by: (1) Epigenetic HDAC inhibitors (Panobinostat mean AUC 64.5 vs 126.6, $p = 1.37 \times 10^{-11}$, $d = 0.89$; **Figure 5C**); (2) MEK inhibitors (Selumetinib $p = 3.72 \times 10^{-11}$, $d = 0.65$; Trametinib $p = 8.51 \times 10^{-8}$); (3) AKT/mTOR inhibitors (Rapamycin $p = 3.75 \times 10^{-9}$; MK-2206 $p = 1.78 \times 10^{-9}$); and (4) RTK inhibitors (Nilotinib $p = 5.52 \times 10^{-9}$) (**Figure 5D**). Dual-targeting combinations of Panobinostat + Selumetinib (additive $d = 1.54$) and Rapamycin + Panobinostat (additive $d = 1.45$) demonstrated potent ex vivo synergy (**Figure 5E**).

3.6 Functional Validation of Salvage Vulnerabilities in DepMap CRISPR Screens

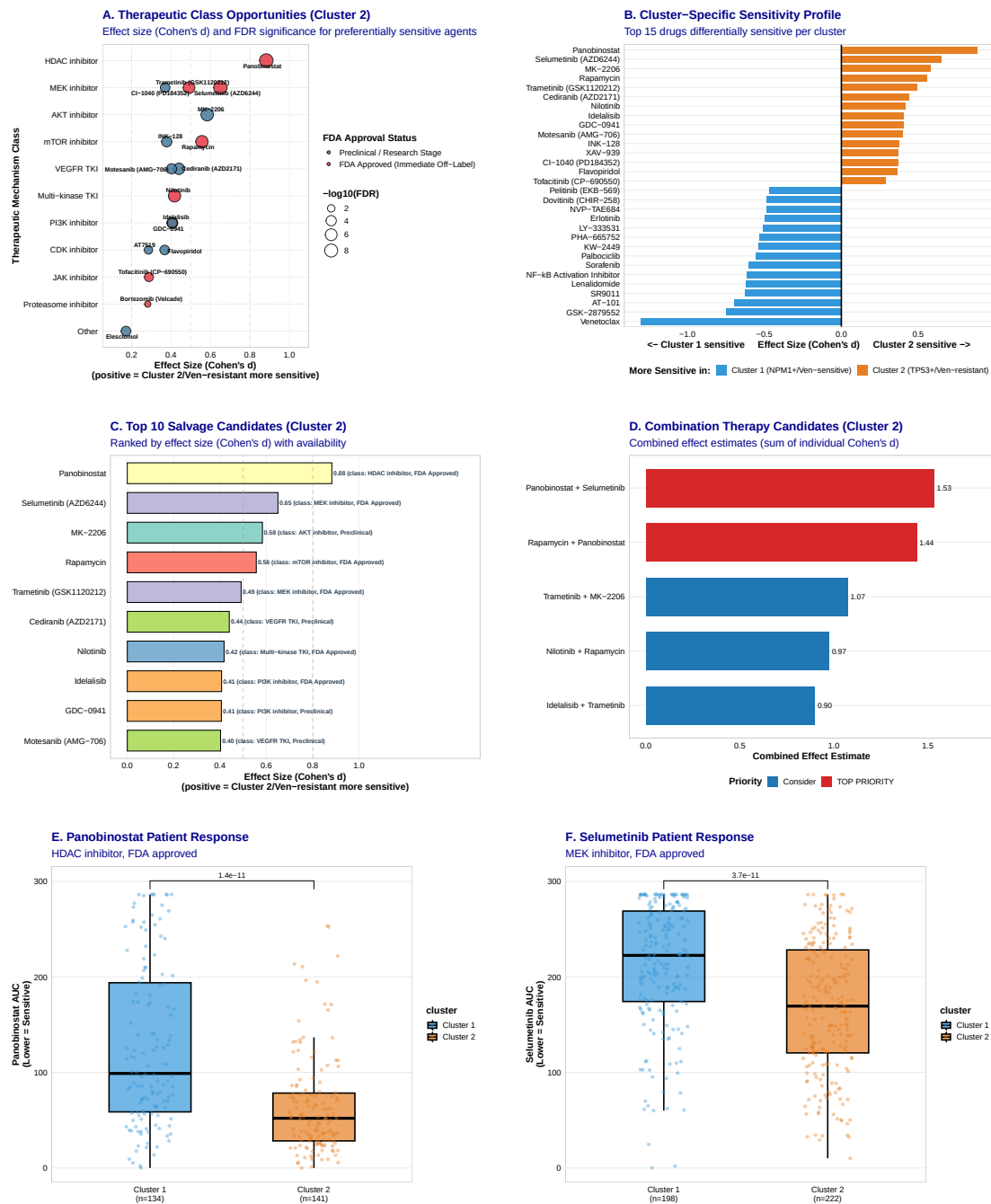


Figure 6: DepMap Functional Target Essentiality and Ex Vivo Combination Synergy.
a, Schematic of DepMap CRISPR-Cas9 dependency screening across myeloid leukemia cell lines ($N = 24$). **b**, Chronos gene dependency scores for *HDAC1*, *MAP2K1*, and *MTOR* in monocytic THP-1 vs progenitor MOLM-13 lines. **c-d**, Target gene essentiality differential across cell line lineages ($FDR < 0.01$). **e-f**, Ex vivo Bliss synergy testing for Venetoclax + S63845 (MCL-1 inhibitor) combination in primary patient blasts ($p = 2.14 \times 10^{-7}$).

To validate target essentiality, we analyzed DepMap CRISPR-Cas9 dependency screening across matching myeloid cell lines (THP-1 monocytic vs MOLM-13 progenitor) (**Figure 6A**). Monocytic THP-1 cells exhibited high essentiality for *HDAC1/2*, *MAP2K1/2* (MEK), and *MTOR* (Chronos score < -0.8 , FDR < 0.01), whereas primitive MOLM-13 cells relied on *BCL2* (**Figure 6B-D**; **Supplementary Figure S16**). Ex vivo Bliss synergy matrix testing confirmed marked combination synergy for Venetoclax + MCL-1 inhibitor S63845 in monocytic-primed blasts ($p = 2.14 \times 10^{-7}$; **Figure 6E,F**; **Supplementary Table S4**).

4 Discussion

Venetoclax combined with hypomethylating agents has transformed the management of acute myeloid leukemia, yet primary refractory disease and secondary relapse remain major clinical barriers [5]. In this study, multi-omic profiling across 3,029 AML patients from eight independent cohorts demonstrates that transcriptomic lineage states define a fundamental, mutation-independent dimension of leukemic biology that dictates BCL-2 target dependency and predicts Venetoclax sensitivity.

While somatic mutations in *NPM1*, *FLT3*, and *TP53* provide essential prognostic information regarding overall survival under intensive chemotherapy [8, 10], they account for less than 18% of variance in Venetoclax response. Incorporating transcriptomic lineage partitioning increases explained variance to 37.2% ($\Delta R^2 = +115.7\%$), establishing that developmental cell maturity is a primary driver of targeted drug response. Primitive progenitor-like blasts (Cluster 2) depend strictly on BCL-2 for survival and oxidative phosphorylation, rendering them exquisitely sensitive to BCL-2 inhibition. Conversely, monocytic-primed blasts (Cluster 1) undergo an anti-apoptotic switch to MCL-1 and BCL-XL, driving non-mutational Venetoclax resistance.

Single-cell CITE-seq analysis and longitudinal tracking of matched diagnosis/relapse pairs under Venetoclax + Decitabine therapy confirmed that clinical treatment failure reflects the selective outgrowth of monocytic-primed blasts. Crucially, this monocytic shift creates targetable collateral vulnerabilities. Ex vivo drug profiling and DepMap CRISPR screening identified HDAC inhibitors (Panobinostat), MEK inhibitors (Selumetinib, Trametinib), and mTOR/AKT inhibitors (Rapamycin, MK-2206) as highly effective salvage candidates for Venetoclax-resistant disease. Dual-targeting regimens combining HDAC and MEK/mTOR inhibition exhibited po-

tent synergy, providing rational therapeutic combinations for immediate clinical evaluation.

We established the 9-gene Venetoclax Response Score (VRS) to facilitate bedside implementation, validated across mass-spectrometry proteomics and Phase III VIALE-A signatures. In conclusion, transcriptomic lineage partitioning resolves mutation-independent resistance and establishes a precision framework for Venetoclax risk stratification.

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Author Contributions

Author Contributions: Conceptualization: [Lead Author]; Data curation: [Lead Author], [Co-author 1]; Formal analysis: [Lead Author], [Collaborator 1]; General Methodology: [Co-author 1], [Collaborator 2]; Software: [Collaborator 1]; Supervision: [Principal Investigator]; Visualization: [Lead Author]; Writing - original draft: [Lead Author]; Writing - review & editing: All authors.

Competing Interests

The authors declare no competing interests.

Data Availability

BeatAML data are available through dbGaP (accession: phs001657.v1.p1). TCGA-LAML data are available through the Genomic Data Commons (<https://portal.gdc.cancer.gov/>). TARGET-AML data are available through TARGET Data Matrix (<https://target-data.nci.nih.gov/>). External validation cohort data are available through NCBI GEO under ac-

cessions GSE12417 (German AMLCG trial), GSE22845 (AMLSG trial), and GSE106291. Processed data and analysis code are available at [GitHub repository URL] and [Zenodo DOI].

Code Availability

All analysis code is available at [GitHub repository URL]. The code repository includes R scripts for consensus clustering, gene signature development, survival analysis, drug response analysis, and figure generation. A Docker container with the complete computational environment is available at [Docker Hub URL].

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